A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 May 2026

Dr Jacob Sanders
Inner North Medical Clinic

Dear Dr Sanders,

Re: Mr Stephen Naimo
DOB: 17/11/1986 Phone: 0409825100
Address: 1 Echersall Street, BRUNSWICK, VIC, 3056
Email: stephen.naimo@gmail.com

Summary

- **Pouchoscopy revealing non-inflammatory ulceration within the pouch.**
- **Histology confirming no evidence of active inflammation or malignancy.**
- **Surveillance pouchoscopy recommended in five years.**
- **Action required: Please facilitate antibiotic therapy for future protracted symptomatic episodes of pouchitis as outlined below.**

I had the pleasure of reviewing Stephen following his recent pouchoscopy. As we discussed during our consultation, I noted a couple of ulcers within the pouch that were macroscopically similar to those seen during his previous assessment. I am pleased to report that the biopsy results were entirely reassuring, revealing no evidence of active inflammation, malignancy, or dysplasia. As I explained to Stephen, these ulcer-like findings are relatively common in pouch surveillance. While they can appear significantly inflamed, the lack of histological correlation suggests a multifactorial aetiology. It is often theorised by our surgical colleagues that such findings may relate to local vascular supply and pouch mechanics rather than true inflammatory activity. Given the benign histology, these findings do not require specific treatment or changes to his current management.

We also discussed his history of pouchitis, including a prolonged episode last year that was managed symptomatically. Stephen noted that antibiotics have been used in the past but were not utilised during this most recent flare. While I understand there can be some hesitation regarding antibiotic use, I believe that for protracted symptomatic episodes of pouchitis, a course of Metronidazole or Ciprofloxacin remains the most appropriate strategy.

Impression and Plan I have provided Stephen with an electronic prescription for a seven-day course of antibiotics to keep on hand should his symptoms recur. If he experiences a future episode of pouchitis that does not resolve promptly, I would be grateful if you could facilitate a similar antibiotic approach. In view of his stable pouch appearance and normal histology, I have recommended a surveillance pouchoscopy in five years' time. I have asked Stephen to contact me if he develops any concerning symptoms in the interim.
Thankyou for re-referring Stephen back to my care. Histology below.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

NAIMO, STEPHEN : SVP 619592

DOB: 17/11/1986 Age: 39 years

1 Eckersall St, Brunswick 3056

Results from 24/04/2026 ..... Episode# 26P264849 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010794 **CLINICAL NOTES** J pouch Ileum = ulcer. Afferent limb = ulcer. Efferent limb = ulcer Cuff = normal but scarred. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Ileum". Two mucosal fragments measuring 5-6 mm. A1. 2. Specimen labelled "Afferent limb". Two mucosal fragments measuring 3-4 mm. A1. 3. Specimen labelled "Efferent limb". A mucosal fragment measuring 5 mm. A1. 4. Specimen labelled "Cuff". Two mucosal fragments measuring 2-6 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** **1. Ileum - Terminal ileum mucosal biopsies with no significant pathological features. No parasites or granulomas are identified. No evidence of dysplasia or malignancy. 2. Afferent limb - Terminal ileum mucosal biopsies with no significant pathological features. No parasites or granulomas are identified. No evidence of dysplasia or malignancy. 3. Efferent limb - Terminal ileum mucosal biopsies with no significant pathological features. No parasites or granulomas are identified. No evidence of dysplasia or malignancy. 4. Cuff - Terminal ileum mucosal biopsies with some hyperplastic regenerative changes and no other pathological features. No parasites or granulomas are identified. No evidence of dysplasia or malignancy.** Pathologist: Dr Cesar Salinas-LaRosa

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